

Xana Plus App — Team Discussion Guide

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AT A GLANCE

- **Goal for the meeting:** agree what version 1 of Xana Plus is, so backend work can start against a fixed scope.
- **Where we are:** all 53 customer screens are designed and run on a phone, but on sample data. No backend exists yet.
- **The tension:** the screens describe a grocery-and-pharmacy delivery app; the PRD describes a national health super app.
- **The pharmacy gate:** it cannot launch without PPB registration as a digital pharmacy and a pharmacist-side workflow, which is not built.
- **Five decisions** unblock the backend (section 8). Please read sections 4 and 6 before the meeting.
- **Questions** are split into ones for the team and ones for IT/dev to take away (section 7).

PROPOSED · FOR AGREEMENT

Version 1: shopping and pharmacy, done properly

The 53 designed screens made real, plus the pharmacist side and the PPB fixes. Everything else in the PRD waits.

SHOPPING AND DELIVERY

- Live catalogue, prices and stock from Business Central
- Browse, search and basket across departments, with bulk pricing
- Out-of-stock choices, live substitutions, order tracking
- Returns and refunds to M-Pesa or Club credit

PAYMENTS AND ACCOUNT

- Sign-in by phone with a real SMS or WhatsApp code
- M-Pesa, card and cash on delivery; eTIMS receipt on every sale
- Xana Club points: earn and spend
- Addresses with a live delivery-zone map

PHARMACY

- OTC shopping and prescription upload
- Pharmacist workbench: review queue, approve, clarify, hold, decline
- Pharmacy status model; no Rx dispatch without a decision
- Repeat and refill requests, refill reminders
- Real pharmacist chat

PPB COMPLIANCE

- "About and verify" page, policies, Health Safety Code
- "Not for online sale" flag; no promotions on medicines
- Sealed packaging and ID or OTP check at delivery
- "Report a side effect" and full audit trail

IF CAPACITY ALLOWS (V1 OR V2)

Family profiles, dose reminders, medicine info cards, Swahili

NOT IN V1

Clinic booking, Wholesale accounts, telehealth, labs, SHA and insurance, paid membership, health record, community

1 Purpose and agenda

Four inputs were reviewed, and they now pull in different directions:

- **Screen pack** — “Xana Plus — What the App Does”, 23 Sep 2026, 10 pages
- **Brenda’s feedback** — pharmacy flow, ease of use, delivery and support
- **Developer review and pharmacy requirements** — 27 Sep 2026, 5 pages
- **Guidelines on online pharmacy set-up** — a summary of the draft 2026 PPB Digital Pharmacy Guidelines, 6 pages
- **PRD** — “Xana Plus Super App”, 23 Sep 2026, 19 pages

This guide summarises those documents for discussion; it is not a legal or regulatory opinion.

Proposed agenda — 90 minutes

1 · 10 min Where we are Screens done, no backend	2 · 20 min Scope Delivery app or super app; what is in V1	3 · 20 min Pharmacy compliance PPB must-haves
4 · 15 min Pharmacy workflow The pharmacist workbench	5 · 15 min Feature requests Sort into V1 and later	6 · 10 min Decisions Owners and next steps

2 What the screens cover today

The screens deliver a customer-side shopping and pharmacy app; nothing behind them exists yet. A customer browses without an account, signs in by phone and SMS code at checkout, pays by M-Pesa, and tracks the order to the door. Pharmacy items share the grocery basket, with a pharmacist check before anything Rx ships.

Area	What is designed	What is fake today
Shopping	Home, six departments (Retail, Wholesale, Deli, Pharmacy, Groceries, Liquor), search, product page, barcode scan	Fixed sample catalogue; scan is simulated
Basket and payment	Bulk pricing, out-of-stock preference, Xana Club points at 10 points = KES 1, M-Pesa, cash on delivery, card, payment-failed screen	Any number signs in, code is 123456; M-Pesa is a timer
Orders and delivery	Four-stage tracking, live substitutions, item on hold, return and refund to M-Pesa or Club credit	Orders live on the phone only
Pharmacy	Upload prescription, My prescriptions, repeat orders, Ask a pharmacist, chat, six clinical services, booking	Sample pharmacist replies; no review queue
Account	Club tier and points, addresses with delivery-zone pin, Savings and spending page	Map is a drawing

Privacy is already designed in: medicine names never show on shared or summary screens, and the PPB licence shows on every pharmacy screen. The screen pack lists six backend pieces before launch: real SMS sign-in, live catalogue from Business Central, live M-Pesa, a central order system, a pharmacist tool, and app-store listings.

3 What stakeholders asked for

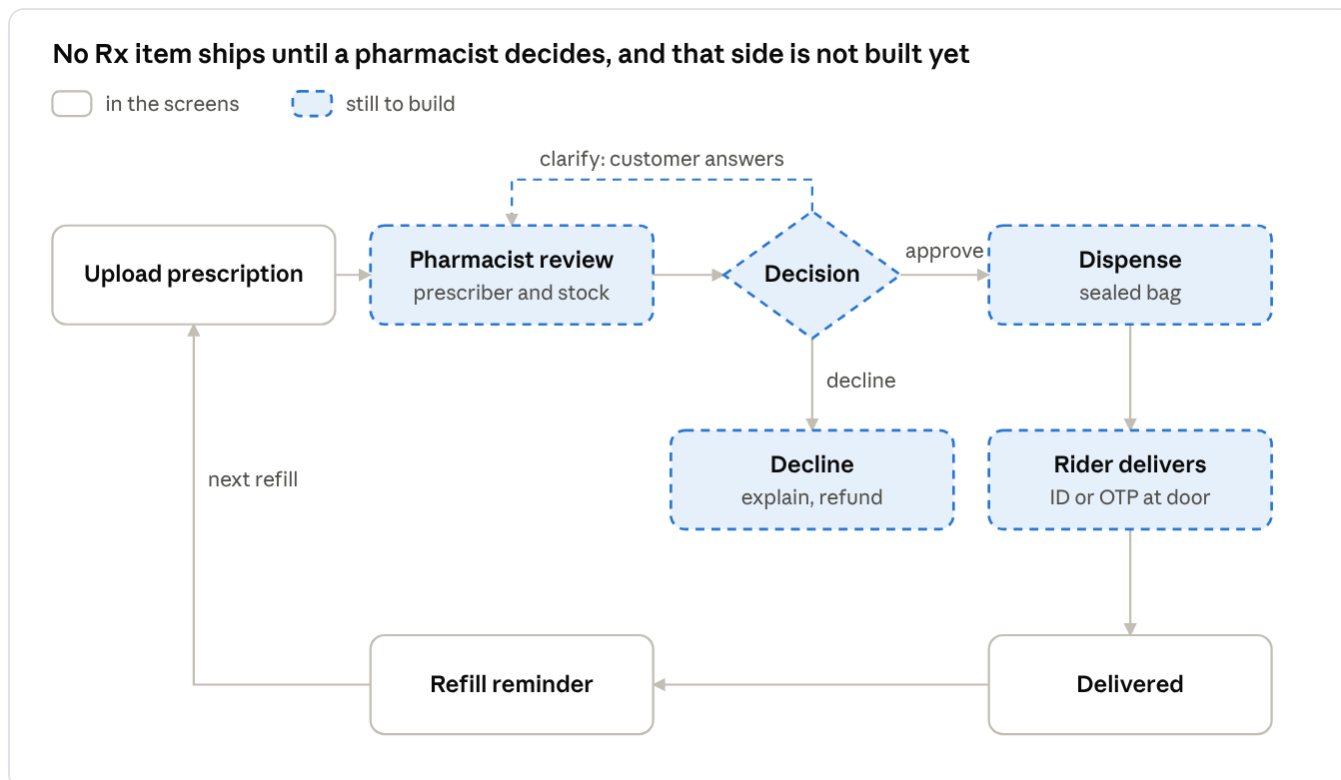
The feedback agrees on one point: the pharmacy is the differentiator and needs its own workflow, not a tab in a grocery app. Most of Brenda's suggestions are already in the PRD; three of them raise compliance questions.

Brenda's suggestions

Suggestion	Also in	Note for discussion
Family profiles	PRD (core), developer review (P2)	Needs a consent model for adult dependants and the age-18 handover
Dose reminders	PRD (taken/skipped logging)	Low backend cost; strong daily-use hook
Medicine info cards	PPB guidelines require counselling on dosage, side effects, storage	Compliance need , not a nice-to-have. Who writes and reviews the content?
After-hours prescription message	—	Hours conflict: screens say 7am–10pm, Brenda says 7am–9pm
Discreet packaging option	PPB: tamper-evident packaging, confidentiality in transport	Make it the default for all pharmacy items, not an option
Digital itemised receipts	PRD (eTIMS on every sale)	Supports insurance reimbursement claims
Swahili, voice search	PRD (Swahili and English at launch)	Screens are English only today
Lite mode	PRD (3G, offline, PWA)	
Text size, dark mode	PRD (WCAG 2.1 AA, large text)	
Rider name, photo, call button	PRD (rider app)	Needs a rider app or partner; not designed yet
Support chat, in app or WhatsApp	PRD (shared support chat)	Who staffs it, and what hours?
Ratings for products and deliveries	—	PPB bans testimonials for medicines : rate deliveries and groceries only

Developer review (27 Sep)

- **Pharmacist workbench is the critical gap.** The customer pharmacy journey is not complete until a pharmacist can receive, review and decide on requests.
- **A pharmacy status model** of nine states: Received, With pharmacist, Clarification required, Approved, Partially approved, Dispensing, Ready, Out for delivery, Completed.
- **P1 items:** workbench and queue, clarification workflow, pharmacy-specific out-of-stock handling, status model, real pharmacist chat.
- **Mixed basket rules:** flag Rx items, explain whether groceries can ship while review is pending, never dispatch Rx before a decision.
- **12 acceptance criteria** before pharmacy launch, ending in an end-to-end customer → pharmacist → store → rider test.



Pharmacy order flow. Only the customer's ends of the flow exist today; every step where a pharmacist or rider acts is still to build.

4 PPB must-haves against the designs

The pharmacy cannot launch without PPB registration as a digital pharmacy, and several screens need changes to meet the draft guidelines. The biggest design conflicts are in advertising: the app's deals, discounts and branded product pages collide with what the guidelines allow for medicines.

Requirement (draft 2026 PPB guidelines)	Where the designs stand	Gap to close
PPB registration of the online pharmacy, linked to a licensed physical premises	PPB licence shown on pharmacy screens	Which branch is the licensed premises? Has registration been applied for?
Superintendent pharmacist appointed	Not covered	Name the person; they own digital system oversight too
Platform shows name, licence, address, responsible pharmacist, PPB Health Safety Code, privacy policy, T&Cs, complaints process, returns policy	Licence only	Add a pharmacy "About and verify" screen and the policy pages
Verify prescriber identity and registration; validate script; screen for duplicate dispensing	Photo upload, pharmacist review	How does a pharmacist verify a paper script's prescriber, and stop the same script being filled in-store and online?
Prohibited online: controlled drugs, drugs needing therapeutic monitoring (digoxin, lithium, carbamazepine), hospital-only, health-worker-administered	Not covered	A "not for online sale" flag per product, enforced in search, basket and repeat orders

Requirement (draft 2026 PPB guidelines)	Where the designs stand	Gap to close
No direct advertising of Rx medicines; no branded names or product images publicly; no flash sales, discounts or testimonials	Home deals, promotions, member pricing, product ratings	Keep promotions off medicines. Hide Rx product pages until a prescription is linked? Confirm with the superintendent
No targeted medicine ads from health data	"Your regulars" reorder, savings page	Keep reorders, but no medicine promotions driven by purchase history
Pharmacist consultation by message, audio or video; counselling on dose, side effects, storage	Chat by WhatsApp or app	Is WhatsApp a "secure channel" under the guidelines? Audio or video not designed
Traceability from dispensing to delivery; verify patient identity at delivery; tamper-evident packaging	Four-stage grocery tracking	Delivery OTP or ID check for pharmacy orders; sealed bags
Cold chain: monitor temperature, under 8 hours in transit	Not covered	Flag cold-chain items; limit to express delivery
Records and audit trails for prescriptions, consults, dispensing, deliveries; pharmacovigilance reporting	Not covered	Add a "report a side effect" entry point; audit log in the backend
Data Protection Act 2019, Digital Health Act 2023, encryption, role-based access, breach process	Not covered	DPIA and ODPC registration before launch
QA framework, quarterly audits, KPIs, SOPs	Not covered	Operations work, not app work, but it gates launch

The guidelines summary is based on a **draft**. We should confirm whether the final version has been published and what has changed.

5 What the PRD proposes

The PRD is a full vision, not a version 1: it explicitly leaves phasing and budget out. It describes about 12 modules; the screens cover roughly two of them.

The PRD's own principles are worth adopting now: one identity and one record across modules, M-Pesa first, Business Central as stock and price master, and a licensed human accountable for every clinical action.

The phase column is a starting proposal for the meeting to change, not a decision.

Module	In the screens?	Main dependency	Proposed phase
Grocery, Deli, Liquor, Retail shopping and delivery	Yes	Business Central catalogue, order system, riders	V1
Pharmacy: OTC, prescriptions, refills, pharmacist chat	Yes, customer side only	PPB registration, pharmacist workbench	V1
Payments: M-Pesa, card, eTIMS receipts	Yes	Existing Daraja and eTIMS integrations	V1
Xana Club points	Yes	Existing loyalty system	V1
Family profiles and dose reminders	No	Consent model	V1 or V2
Clinical services booking (in-store)	Yes	Branch scheduling	V2

Module	In the screens?	Main dependency	Proposed phase
Wholesale and business accounts, group buying	Partly (Wholesale department)	Credit checks, eTIMS B2B invoices	V2
Telehealth: GP and specialist consults	No	Clinical model decision, KMPDC rules, video vendor	Later
Labs and diagnostics	No	Lab partners, FHIR integration	Later
SHA and insurance claims	No	SHA interfaces unknown; PRD calls it the highest risk	Later
Xana Plus paid membership	No	Pricing; Safaricom recurring payments	Later
Personal health record (FHIR)	No	Data protection, DPIA	Later design the data model for it in V1
Wellness, community, Care Circles, harambee, faith layer, mini-programs	No	Counsel on harambee and chamas; moderation staff	Later

The PRD lists 9 business decisions and 17 items to verify with SHA, PPB, KMPDC, CBK, ODPC and counsel. None of them blocks a V1 limited to shopping and pharmacy, except the PPB rules.

6 Where the documents disagree

The documents disagree on nine points that change what gets built. The order-and-payment sequence matters most, because it shapes the backend order model.

Topic	One document says	Another says	Why it matters
When the customer pays for Rx items	Screens: pay at checkout, review follows	PRD: placed → pharmacist review → paid	Pay first means refunds on every declined script; review first means a second payment step
Order status model	Screens: 4 stages for orders, 4 for prescriptions	Developer review: 9 pharmacy states; PRD: 8 states plus cancelled and returned	One status model is needed before the order system is built
Out-of-stock handling	Screens: customer picks replacement, refund or call	Developer review: pharmacy substitution must be pharmacist-controlled	Grocery substitution rules cannot apply to medicines
Pharmacist review hours	Screens: 7am–10pm, about 30 minutes	Brenda: 7am–9pm	Promise only what the rota can staff
Product scope	Screens: Liquor, Deli, Groceries departments	PRD: health, wellness and household; no liquor or fresh food	Liquor next to a pharmacy in a health-branded app; age checks at delivery
Name “Xana Plus”	Screens: app name; rewards are “Xana Club”	PRD: “Xana Plus” is also the paid membership	PRD lists this as an open brand decision
Deals and discounts	Screens: weekly deals, member pricing, points on everything	PPB: no discounts or flash sales on medicines	Promotion engine must exclude pharmacy lines

Topic	One document says	Another says	Why it matters
Product ratings	Brenda: rate products	PPB: no testimonials for medicines	Ratings limited to non-medicine products and deliveries
Sign-in strength	Screens: phone and SMS code only	PRD: health record with consent; shared phones	Is an SMS code enough to open prescriptions on a shared phone?

No document settles two things: who the riders are (own staff, a partner, or store staff), and whether app orders are rung through the existing POS, so they reuse today's Business Central, eTIMS and loyalty paths instead of new ones.

7 Questions pending

The open questions split by who should answer them. **The team** settles business, pharmacy and operations questions in the meeting. **IT/dev** takes the technical ones away, comes back with answers, and flags anything that needs a business call.

For the team to decide

Business, pharmacy and operations. Anything left open becomes an action with an owner.

Scope and product

- ☐ Is V1 shopping plus pharmacy only, with the rest of the PRD phased later?
- ☐ Do Liquor and Deli stay in the same app as the pharmacy?
- ☐ Which branches launch first, and which one is the licensed premises for the online pharmacy?
- ☐ Is "Xana Plus" the app, the paid membership, or both? How does it sit beside Xana Club?
- ☐ Swahili at launch, or English first?

Operations and delivery

- ☐ Own riders, a delivery partner, or store staff?
- ☐ Which cold-chain items can we deliver, and within what time?
- ☐ Who staffs general support chat, and during what hours?

Ownership

- ☐ Build in-house, with a vendor, or a mix? Who owns the roadmap?
- ☐ Who owns the DPIA and ODPC registration?

Pharmacy and compliance

- ☐ Who is the superintendent pharmacist for the online pharmacy, and when do we apply for PPB registration?
- ☐ Is the PPB guideline final, or still a draft?
- ☐ Do customers pay before or after pharmacist review?
- ☐ What are the review hours, and how many pharmacists cover the queue?
- ☐ How does a pharmacist verify a paper prescription's prescriber?
- ☐ Who keeps the "not for online sale" product list (controlled, monitored, hospital-only)?
- ☐ Can Rx product pages show brand names and photos, or only after a prescription is linked?
- ☐ Do points, member prices and promotions apply to medicines at all?
- ☐ Is WhatsApp acceptable for pharmacist chat, or in-app only?
- ☐ Who writes and reviews medicine info cards?
- ☐ At delivery, is an OTP enough, or must the rider see ID?

For IT/dev to decide

Technical calls. Bring a recommendation back to the team; escalate only where it changes cost or scope.

Architecture and integration

- ☐ Does the pharmacist workbench extend the existing Collabmed pharmacy module (prescriptions, refills, controlled-drugs register already live), or is it a new tool?
- ☐ Do app orders go through the existing POS, Business Central, eTIMS, M-Pesa and loyalty integrations?
- ☐ How do app orders reach a store to be picked, and how do riders see them?
- ☐ How fresh can app stock be against Business Central? The PRD target is 15 minutes.
- ☐ Where are “not for online sale” and “no promotions” flags held: Business Central item data or our catalogue?

App, data and security

- ☐ Android first, or Android and iOS together? Native or cross-platform?
- ☐ One status model for grocery and pharmacy orders: how do the 9 pharmacy states map onto it?
- ☐ Is an SMS code enough to open prescriptions on a shared phone, or does pharmacy need a PIN?
- ☐ Can we detect one prescription filled both in-store and online?
- ☐ Hosting, data residency, encryption and audit logging for health data
- ☐ How much lite mode and offline support goes into V1?

8 Decisions to leave the room with

Five decisions unblock backend work; everything else can follow. Suggested starting points are for debate, not settled positions.

Decision	Options	Suggested starting point	Who decides
V1 scope	Screens as designed; screens plus family profiles and reminders; PRD modules	Screens as designed, plus the PPB fixes and pharmacist workbench	Team
Rx payment timing	Pay at checkout and refund on decline; review first, then pay	Review first for Rx-only orders; groceries pay and ship at once	Team
Pharmacy status model	Screens' 4 states; developer review's 9	Developer review's 9 states, shown simply to the customer	IT/dev with pharmacy input
Pharmacist workbench	Extend Collabmed pharmacy module; new tool	Extend the existing module; pharmacists already use it	IT/dev
Owner for PPB registration and superintendent	—	Name one person at the meeting	Team

Next steps

- ☐ Send the agreed V1 scope to the team
- ☐ Confirm PPB guideline status and start registration
- ☐ Draft the pharmacy status model and mixed-basket rules
- ☐ Rework screens for the PPB gaps: verify-pharmacy page, policies, promotions off medicines, delivery identity check
- ☐ Map V1 backend onto existing Business Central, eTIMS, M-Pesa and loyalty integrations
- ☐ Book a follow-up to review the updated screens